

CONTEMPORANEOUS INCIDENT LOG NOTES

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Period Covered: Late August 2025 – 3 October 2025

Purpose: Attachment to Formal Grievance (submitted 6 October 2025) and evidence for Employment Tribunal proceedings. ACAS Early Conciliation Ref: R260632/25. Access to Work Ref: 5RRLKN95.

Confidentiality Notice: Strictly Private & Confidential — Without Prejudice Save as to Costs. This log is a contemporaneous, factual record prepared in accordance with ACAS guidance. It documents events, witnesses, evidence and observed impacts relevant to a formal grievance alleging disability discrimination, harassment, victimisation, breaches of data protection and health & safety failures by named managers (RJ and SD).

Key / Abbreviations

- **RJ** — Line manager (named)
- **SD** — Plant / acting shift manager (named)
- **MIB, NAH, MOC, ARS, TJA, RS, HRK, TNO, ADY, SAV, RB, CGS, DAR, SJ, ATA, SLS, KDY, MIC, RSM** — Colleagues / witnesses (full names available to investigator)
- **UKSL 425** — Internal health-incident reporting form (Sickness Absence process)
- **OH** — Occupational Health
- **BPA** — Relevant production area (Building 228 referenced)
- **Suite 2 / Suite 1 / Building 640 / Building 228** — Locations on site

Summary of Core Allegations (short) Since a confidential disclosure of a long-term mental health condition and medication side-effects on **13 September 2025**, I experienced a sustained pattern of: insecure recording/sharing of my special-category health data, repeated intrusive public questioning and shaming, disproportionate probation extension/targets, failure to refer to OH following repeated health incidents, and escalating victimisation when I attempted to raise concerns. These incidents cumulatively caused a significant deterioration in my mental health, culminating in NHS crisis team involvement on **3 October 2025**.

Detailed, Chronological Incident Entries (ACAS standard)

Note: each entry follows the same structure: **Date / Time / Location → Factual description → Actions taken at the time → Immediate impact observed → Policy / legal issues (brief) → Evidence & Witnesses.**

Late August – Early September 2025 (various shifts — 2–3 occasions) Date/Time:

Multiple dates (late Aug – early Sep 2025). Exact timestamps recorded in UKSL 425 forms and my contemporaneous notes. **Location:** Building 228 (BPA area) and Building 640 (office/hot-desk). **Factual description:** I experienced episodic diarrhoea attributed to medication side-effects. On 2–3 occasions I reported incidents to SD via the UKSL 425 reporting process (per policy). SD acknowledged the reports but did not make a referral to Occupational Health or discuss reasonable adjustments. Over the following days managers and supervisors (including RJ) began privately and publicly questioning my health and attendance; I was later told by RJ/SD that I had been “off-plant every other day.” **Actions taken at the time:** Reported each episode via UKSL 425; continued to attend duties where able; made contemporaneous notes after each incident.

Immediate impact: Heightened anxiety, emergence/worsening of insomnia (sleep often <4 hours), loss of appetite (skipping meals), beginning of social withdrawal and reduced engagement at work. **Policy / legal issues:** Sickness Absence Policy obligations to consider OH referral and reasonable adjustments; Equality Act 2010 (discrimination arising from disability); HSWA s.2 (welfare duty). **Evidence:** UKSL 425 report entries; contemporaneous notes (dated); request to IT for deleted Teams entries; GP letter (Dr Tamkin Abas, 30 Sep 2025). **Witnesses:** SD; RJ; on-shift supervisors (names available).

8 September 2025 — c.09:00 (Morning) Location: Building 640 (office) **Factual**

description: RJ returned from leave, introduced himself as my line manager and arranged a probation review for 13 September. RJ commented on my “nervous” appearance in front of colleagues. **Actions taken:** I continued with assigned buffer

preparation tasks in the BPA area with ARS, TGA and SAV. I made contemporaneous notes documenting RJ's comment. **Immediate impact:** Increased unease leading to anxiety and fear of job insecurity; onset of panic symptoms that day. **Policy / legal issues:** Conduct Policy (bullying/harassment) – precursory behaviour consistent with harassment (EqA s.26). **Evidence:** Contemporaneous note (8 Sep 2025); shift schedule showing presence of ARS/TGA/SAV. **Witnesses:** ARS, TGA, SAV, RJ.

9 September 2025 — c.16:30 (Afternoon) Location: Building 228 (BPA) **Factual description:** HRK reported colleagues' concerns that I "looked nervous" and suggested this affected performance. Separately, TJA advised RJ I had been "unlocatable" during a lunch period; this was later reframed as an allegedly excessive lunch (~40 minutes vs. typical 30). I had in fact informed the on-shift supervisor before leaving for lunch. Other colleagues (ARS, TJA, TNO) routinely took similar breaks but were not subject to the same scrutiny. **Actions taken:** I explained the reason for my nervousness and that I had informed a supervisor prior to leaving; I requested discretion. I emailed TJA / ARS / TNO for feedback on my performance (no substantive responses). **Immediate impact:** Humiliation, increased anxiety, panic episodes, worsening sleep, reduced appetite, beginning to withdraw socially and professionally. **Policy / legal issues:** Differential treatment potentially amounting to discrimination (EqA s.15) and harassment (EqA s.26); conduct policy breach if targeted. **Evidence:** Contemporaneous notes (9 Sep); Team rota/schedule; emails requesting feedback (no reply). **Witnesses:** HRK, TJA, ARS, TNO, RJ.

12 September 2025 — c.10:00 (Morning) Location: Building 640, Suite 2 **Factual description:** Performed filtration tasks alongside MOC and CGS; no performance issues observed. I later emailed MOC requesting feedback; no reply. **Actions taken:** Completed assigned tasks and documented activity in contemporaneous notes. **Immediate impact:** Continued anxiety and frustration at lack of peer feedback; feeling isolated. **Policy / legal issues:** Contextual evidence undermining later managerial claims of unreliability. **Evidence:** Contemporaneous notes; schedule records; email sent to MOC. **Witnesses:** MOC, CGS.

13 September 2025 — c.14:00 (Probation review; morning duties also performed) Location: Building 640 (office) / Suite 2 duties earlier that day **Factual description (core event):** During the probation review with RJ I disclosed my long-term mental health condition and medication side effects (including intermittent incontinence / urgent toilet needs). RJ stated he keeps logs on direct reports; I was not told how or where this would be stored. That same day RJ recorded sensitive health information in an **unprotected Excel worksheet** on his work login / shared drive. RJ also extended my

probation to six months and imposed notably onerous and specific targets (95% CSOD completion, additional BPA demonstrations/sign-offs) which I was told peers were not required to meet. **Actions taken:** I disclosed confidentially and requested privacy. I documented the meeting in contemporaneous notes and sent a follow-up email requesting clarification of probation targets (no substantive reply). **Immediate impact:** Acute vulnerability; increased humiliation and anxiety; sleep disruption; appetite loss; further social withdrawal; concern about data confidentiality. **Policy / legal issues: UK GDPR (Article 9)** — unlawful processing/storage of special-category health data if no lawful basis/secure processing; Lonza Conduct/Confidentiality policy breach; Equality Act (discrimination / failure to adjust). **Evidence:** Contemporaneous notes; request for IT to preserve RJ's Excel file (file path/metadata where available); probation documentation (extension notice); follow-up email to RJ/NAH. **Witnesses:** RJ; NAH; MOC; CGS; RSM (if present).

14 September 2025 — c.11:00 (Morning) Location: Building 640 (Suite 2) **Factual description:** Performed filtration and buffer tasks with MOC, CGS, MIC, DAR without incident. Requested feedback by email from MIC; no reply. **Actions taken:** Completed tasks and logged activity. **Immediate impact:** Continued stress about lack of managerial/peer support. **Policy / legal issues:** Contextual corroboration of competence despite managerial claims. **Evidence:** Contemporaneous notes; schedule records; email to MIC. **Witnesses:** MOC, CGS, MIC, DAR.

17 September 2025 — c.08:30 (Morning) Location: Building 640 (hot-desk area / change area / suites) **Factual description:** Reported an episode of diarrhoea via UKSL 425 to SD and RJ. In the change area RJ questioned me about my toilet frequency, referencing a comment attributed to MIB that it was “concerning that a young man like me needs to use the toilet early in the morning.” RJ also raised allegations I had been “unlocatable.” No referral to OH was made despite repeated incidents. **Actions taken:** Followed reporting procedure (UKSL 425), responded to RJ's questions and disputed inaccurate claims. Documented exchanges in contemporaneous notes. Emailed MIB requesting clarification; received no reply. **Immediate impact:** Embarrassment, humiliation, anxiety spike, worsening insomnia and panic symptoms; loss of confidence in management; increased social withdrawal. **Policy / legal issues:** Harassment under EqA s.26; discrimination arising from disability (EqA s.15); Sickness Absence Policy / OH referral ignored; potential data confidentiality breach if others were informed. **Evidence:** UKSL 425 entry; contemporaneous notes (17 Sep); Teams messages (if any); email to MIB. **Witnesses:** RJ, SD, MIB.

22 September 2025 — c.10:00 (Morning) Location: Building 640 (Suite 2) **Factual**

description: Completed assigned tasks with RB, CGS, DAR. Requested feedback from RB by email; no substantive reply. Managerial scrutiny continued in parallel. **Actions taken:** Performed duties; logged activity. **Immediate impact:** Ongoing anxiety and sense of isolation. **Policy / legal issues:** No immediate new breach; context for competence. **Evidence:** Contemporaneous notes; schedule; email to RB. **Witnesses:** RB, CGS, DAR.

23 September 2025 — c.09:30 (Morning) Location: Building 640 (Suite 2) **Factual description:** Performed filtration tasks with CGS, DAR, SJ, ATA. Again requested feedback by email; no reply. **Actions taken:** Completed work; documented task completion. **Immediate impact:** Continued anxiety and demoralisation from lack of support. **Policy / legal issues:** Contextual support for later claims of differential treatment. **Evidence:** Contemporaneous notes; emails. **Witnesses:** CGS, DAR, SJ, ATA.

26 September 2025 — c.06:20 (Start of shift) & c.17:30 (Evening) Location: Building 640 (open-plan office / smoking area) **Factual description:** **Morning:** I arrived approximately 20 minutes late for shift. SD publicly admonished me in the open-plan office with disciplinary language, in front of colleagues, and emphasised “common courtesy.” **Evening:** SD approached me in the smoking area and asked “Are you feeling OK?” and questioned my aptitude, in a manner implying prior knowledge of my health condition. Two colleagues (TNO, ADY) who had been late on other occasions were not publicly admonished. No OH referral followed a report I had made earlier that day. **Actions taken:** Responded to SD in the moment, explained the reason for lateness (reported diarrhoea earlier that day), logged the interaction in contemporaneous notes, emailed TNO/ADY requesting feedback (no reply). **Immediate impact:** Public humiliation, severe distress, immediate spike in anxiety and panic symptoms, insomnia worsened. Felt singled out and publicly shamed. **Policy / legal issues:** Possible harassment and victimisation (EqA ss.26 & 27); breach of dignity at work and bullying per Conduct Policy; confidentiality concerns re health data. **Evidence:** Contemporaneous notes (26 Sep); Teams messages; witness statements available; GP letter referencing stress. **Witnesses:** SD, TNO, ADY, SAV, open-plan office staff.

27 September 2025 — throughout shift; recorded end-of-shift c.17:00 Location: Building 640 (office / Suite 1) **Factual description:** Throughout the shift I was asked variations of “Are you OK?” by managers, supervisors and several colleagues. I counted approximately **16 separate instances** during the shift where staff asked about my wellbeing in public or semi-public spaces. I raised my distress to RJ and requested that my concerns be investigated; RJ refused to investigate and responded in a confrontational manner including the phrase “you can do what you want.” RJ also

questioned alleged unlocatability and lateness again. **Actions taken:** Documented each instance with times where possible; raised formal complaint verbally to RJ; sent follow-up communications to RJ and HR requesting investigation (copies retained). Initiated ACAS Early Conciliation (29 Sep). **Immediate impact:** Severe humiliation and emotional distress; panic episodes; further insomnia and appetite loss; increased social isolation and inability to concentrate at work. Felt publicly labelled and ostracised. **Policy / legal issues:** Harassment (EqA s.26) and victimisation (EqA s.27) for raising protected concerns; failure to follow Grievance/GRID obligations in refusing to investigate. **Evidence:** Contemporaneous notes (27 Sep listing 16 queries with approximate times); copies of communications to RJ/HR; ACAS initiation confirmation. **Witnesses:** RJ, SD, SLS, TNO, KDY, SAV, multiple staff.

28 September 2025 — c.17:30 (Evening) Location: Building 640 (office / Suite 2)

Factual description: RJ challenged me about an approved off-plant instance (toilet / shift end), asserting I had not reported. I produced documentation showing supervisor approval. RJ persisted without justification. **Actions taken:** Disputed RJ's claim, provided evidence of supervisor approval, logged the interaction. Emailed NAH for corroboration (no reply). **Immediate impact:** Increased anxiety and fear of disciplinary action; loss of trust in managerial fairness. **Policy / legal issues:** Differential treatment and possible harassment (EqA s.15); failure to properly follow internal reporting/escalation procedures. **Evidence:** Contemporaneous notes (28 Sep); supervisor approval records (where available); email thread. **Witnesses:** RJ, NAH, DAR, CGS, SHC, SAV.

2 October 2025 — c.11:30 (Late morning) Location: Building 640 (hot-desk area)

Factual description: RJ confronted me about completing off-plant CSOD training time, despite assigned tasks having been completed and other team members being similarly off-plant. The trainer acknowledged there was no immediate on-plant work but RJ singled me out. **Actions taken:** Explained the situation to RJ and the trainer; emailed the trainer requesting confirmation of agreed activity (no substantive reply). Logged the event. **Immediate impact:** Humiliation and demoralisation; acute anxiety and difficulty concentrating for remainder of shift. **Policy / legal issues:** Targeting inconsistent with treatment of peers; possible harassment (EqA s.26). **Evidence:** Contemporaneous notes (2 Oct); email to trainer; training schedule. **Witnesses:** RJ; trainer; colleagues.

3 October 2025 — c.18:00 (Evening) Location: Remote (phone contact with NHS;

email to OH) **Factual description:** Due to cumulative workplace stress and a marked deterioration in mental health (panic episodes, severe insomnia, appetite/weight loss), I contacted the NHS mental health crisis team and was triaged for urgent support. I

subsequently emailed Occupational Health requesting an urgent referral. **Actions taken:** Contacted NHS crisis service; followed up with OH via email; documented the crisis contact and symptoms. **Immediate impact:** Psychiatric crisis requiring urgent external intervention; inability to perform work duties safely without immediate remedial measures. **Policy / legal issues:** Health & Safety at Work etc. Act 1974 s.2 (employer duty of care); failure to provide appropriate OH intervention as required under Sickness Absence Policy; potential personal injury claim for psychiatric harm. **Evidence:** NHS crisis team contact details/notes (where available); email to OH (timestamped); GP letter dated 30 Sep referencing deterioration. **Witnesses:** NHS crisis team staff (as appropriate); OH (email records); GP (medical records).

Ongoing / Additional Notes (Post-13 Sep 2025)

- **Probation feedback requests:** I emailed NAH, MOC, MIB, ARS, TJA, RS requesting probation feedback; only RS provided a limited response. This lack of colleague feedback appears to have been discouraged by management. I have preserved copies of those emails.
- **Data protection concerns:** RJ maintained an Excel workbook containing my disclosed health information on a work drive that was not password-protected or otherwise secured (I observed/was shown the workbook). I have requested IT preserve and forensically copy any such file(s).
- **Pattern:** The incidents show a consistent pattern of singling out, public questioning, failure to treat health information confidentially, imposition of disproportionate probation conditions immediately after disclosure, and a persistent failure to refer to OH despite repeated health reports.

Witness Summary (for investigator)

Primary witnesses who were present during or who have direct knowledge of the incidents above (full names and contact details to be provided to investigator on request):

NAH, MOC, MIB, ARS, TJA, RS, HRK, TNO, ADY, SAV, RB, CGS, DAR, SJ, ATA, SLS, KDY, MIC, RSM.

Evidence Summary (for preservation)

Please preserve the following (already requested in formal legal-hold email):

Digital evidence

- RJ's Excel workbook(s) and all versions/backups (OneDrive / local / shared drive).
- Mailboxes (and deleted items) for RJ, SD, named supervisors.
- Teams chat history (including deleted messages) for RJ, SD and named supervisors (dates: 1 Aug – 10 Oct 2025).
- HR system records: probation documentation, UKSL 425 entries, PIP / training sign-offs, performance notes.
- Door-swipe / access logs, Kronos / SAP timesheets for relevant dates.
- CCTV footage (production suites, entry/exit, change areas, smoking area) for the dates referenced.

Physical evidence

- Supervisor handwritten notes, incident reports, printed probation documents.

Medical & third-party evidence

- GP letters (Dr Tamkin Abas — 30 Sep 2025, Mental Health Liaison 14 Oct 2025).
- NHS crisis team contact/triage notes (3 Oct 2025).
- Occupational Health referral emails (if produced) and any OH reports.

Communications

- Emails I sent requesting feedback / addressing incidents (all preserved copies).
- Any replies or lack thereof (showing managerial non-response).

Causation & Impact — summary statement

Each incident recorded above contributed directly to a progressive deterioration in my mental health: heightened anxiety, insomnia, panic episodes, appetite loss and social withdrawal. The refusal to investigate, the public shaming, and the insecure handling of my health data intensified the harm, culminating in the need for urgent NHS crisis care on **3 October 2025**. The pattern of conduct is therefore causally linked to the psychiatric injury I experienced.

Declaration & authenticity

I confirm that these notes were prepared contemporaneously (dates/times recorded at or shortly after the events) and are a true and accurate record to the best of my

knowledge and belief. Originals of electronic notes, emails, UKSL 425 reports, and further documentary evidence are retained and will be made available to the investigator or tribunal.

Signed:

Rehan Arshad Minhas

Date signed: 6 October 2025